

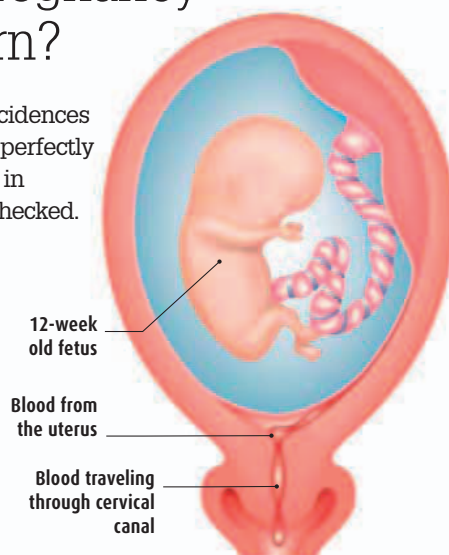
Q Is bleeding in pregnancy always a concern?

Not always. There are lots of unexplained incidences of bleeding early in pregnancies that lead to perfectly healthy, full-term babies. However, bleeding in pregnancy, at any stage, should never go unchecked.

In some cases, women experience bleeding, but the cervix remains closed, the symptoms pass, and the pregnancy continues. This is known as a “threatened miscarriage.”

Doctors used to prescribe bed rest for this, but there’s insufficient evidence that this affects the outcome. If the cervix does go on to dilate, there is really nothing you can do to stop it and a miscarriage is inevitable.

Bleeding in early pregnancy can also indicate an ectopic pregnancy (see p.308). If you have any bleeding, consult your doctor as soon as possible.



Threatened miscarriage In this situation, miscarriage symptoms such as bleeding can occur, but the cervix stays tightly closed, and a healthy pregnancy continues to term.

Q How is an early miscarriage in the first trimester treated?

If you have symptoms such as bleeding, you will be given an early ultrasound. If this reveals that your pregnancy isn’t developing, you have a choice of letting the miscarriage happen naturally, without medical intervention, which is known as “expectant management,” or of having the miscarriage medically managed, either by speeding up the process with drugs, or by surgically removing the pregnancy tissues.

Doctors often advise letting nature take its course for an early miscarriage, so unless you are at risk of a hemorrhage, your doctor will likely recommend that you have a natural, or expectant, management. This gives your body a natural hormonal response to the loss, which is thought to help you adjust physically and emotionally.

It can be hard to predict the exact course of a miscarriage, but at some point you will have bleeding, and, often, painful cramps. You may be asked to take a pregnancy test two to three weeks later to see if hormone levels indicate that the pregnancy has ended. Your doctor will support you and your partner throughout, and you will be offered counseling. If symptoms persist after this time, you will need to see your doctor again. He or she may recommend that

you take a suppository or oral medicine to dilate your cervix to encourage the expulsion of the pregnancy tissues. You will usually begin to bleed within 24 hours of taking the medication, but if you don’t start to bleed, or if a pregnancy test three weeks later indicates that you are still pregnant, you may be referred for surgery.

Surgical removal of pregnancy tissues happens either under a general or (most likely) local anesthetic. In both cases, an instrument called a curette is inserted into your vagina. The curette has a plastic end that gently scrapes away pregnancy tissues, and a hollow tube provides suction to remove the tissues.

Q Is a miscarriage in the second trimester treated differently?

A late-stage miscarriage means that your doctor may suggest that you give birth to your baby.

Although this may sound harrowing, studies show that going through birth can help women cope better long-term both physically and psychologically. You may go into spontaneous labor, but if this doesn’t happen, you will be given a suppository to trigger labor. Your doctor may ask you if you would like to hold your baby. Don’t worry if you don’t want to right

away—or even at all—you must do what feels right for you and your partner. If you would like a photograph of your baby, one of the maternity staff will take one for you.

Q Will I be able to find out what caused my miscarriage?

Sometimes, but not always. It’s often hard to be certain about what caused a miscarriage, and many aren’t investigated. After an early miscarriage, you’re most likely to have a healthy pregnancy next time. For this reason, it’s only when you have had three early miscarriages in a row, known as “recurrent miscarriage,” that your doctor will consider tests to try to find out the cause. You may have tests after one or two late miscarriages. Recurrent miscarriage may be caused by irregularities in the uterus or cervix, recurring genetic issues, or problems with the immune system, endocrine system, the clotting system in the blood, or with your partner’s sperm. Or (most likely of all), there may be no identifiable reason.

During testing, you and your partner may be asked for blood samples to test for genetic causes, doctors will check the placenta if there is one, and for a late miscarriage, you’ll be asked if you would like your baby to have a postmortem. Sometimes the examination of the placenta tells doctors all they need to know. However, it’s important to prepare yourself for the fact that even after investigation, you may never know for certain what went wrong.

Q How long should we wait before trying for another pregnancy?

You should wait until bleeding has stopped before you try to get pregnant again to avoid the risk of infection. It’s also advisable to have

at least one period before trying again, since this makes it easier to date a pregnancy—after the uncertainty of miscarriage, it can be best to avoid confusion in a future pregnancy. Otherwise, it’s up to you both. You may need more time to recover from your loss, or factors such as your age or how long you had been trying to get pregnant may mean you want to try again as soon as possible. Taking folic acid, reducing stress, eating healthily, and monitoring alcohol intake can help maximize your fertility.